CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology immediately for fever, shivering, sudden illness, severe diarrhoea, inability to drink, very little urine or unusual bleeding. New severe chest pain, collapse or major breathing difficulty needs emergency help.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your medicines and what they do

Treatment after colorectal cancer surgery can reduce recurrence risk. For advanced cancer, treatment aims to control the disease. Your team confirms which aim applies.

CHEMOTHERAPY MODULATOR

Folinic acid

Calcium leucovorin



Strengthens fluorouracil action

Folinic acid helps fluorouracil remain bound to an enzyme needed to make deoxyribonucleic acid (the cell's genetic material) (DNA). In this regimen it increases fluorouracil activity and can also increase its unwanted effects. It is not a vitamin supplement to start yourself.

Folinic acid → Stronger enzyme binding → Fluorouracil effect increases

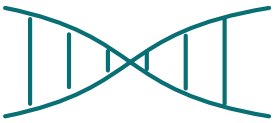
What to watch for

The combined treatment can increase mouth soreness and diarrhoea. Report these promptly. Rare allergic reactions can occur.

CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts cell building blocks

Fluorouracil interferes with DNA and RNA, the instructions cells need to grow. The same drug has different schedules; follow the prescription for this regimen.

Drug enters cell → DNA / RNA disrupted → Growth slows

What to watch for

Report diarrhoea, mouth sores, painful hands or feet and unexpected severe early symptoms. Chest pain needs urgent assessment even after a normal dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) test.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim and duration

Your course

Day 1: folinic acid and fluorouracil injection, followed by a 46-hour fluorouracil pump. Repeat every 14 days. Adjuvant treatment: up to 12 cycles; advanced treatment: continue only while appropriate.

Choices

Discuss the expected benefit, burdens, alternatives and how treatment fits your priorities. You can ask for more time or another explanation before deciding.

Understanding the evidence

Ask which evidence matches your treatment aim.

The NCCP regimen cites fluorouracil/folinic acid experience in colorectal cancer. Benefits differ by setting; no numerical survival estimate from another schedule is assigned to this exact 14-day regimen.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment plan

Fluorouracil + folinic acid every 14 days

14-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ADJUVANT OR ADVANCED COLORECTAL CANCER

Prescribed treatment course

Start | Treatment | Review

Day 1: folinic acid and fluorouracil injection, followed by a 46-hour fluorouracil pump. Repeat every 14 days. Adjuvant treatment: up to 12 cycles; advanced treatment: continue only while appropriate.

The treating team confirms the indication, schedule and duration. Changes are recorded on your own prescription.

BASELINE → TREATMENT → FOLLOW-UP

Tests and checks

Required safety blood tests are separate from the individually prescribed scan or tumour-marker plan.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00660 v2b official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Fluorouracil + folinic acid every 14 days: treatment and checks

Adjuvant or advanced colorectal cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visits	Between visits	Review	After treatment
Blood count / kidneys / liver NCCP 00660	Baseline tests and DPD status	Before each 14-day cycle	Extra tests if unwell or dehydrated	Results guide any delay or change	Follow unresolved abnormalities
Symptoms and medicines NCCP 00660	Review medicines, cardiac history and usual function	Review bowel, mouth, skin and infection symptoms	Use urgent contact plan	Confirm recovery and next dose	Agree recovery care
Scans / tumour markers NCCP 00660	Confirm disease assessment	According to individual plan	New symptoms may bring review forward	Consultant-directed response assessment	Individual follow-up
Heart NCCP 00660	electrocardiogram (ECG) if compromised cardiac function	Report chest symptoms	Chest pain: urgent assessment	Further tests if indicated	Follow any cardiac problem

What each check is for

1 **Confirm your personal plan**

Before starting

Treatment after colorectal cancer surgery can reduce recurrence risk. For advanced cancer, treatment aims to control the disease. Your team confirms which aim applies.

Ask / act: Agree medicines, treatment days, duration and monitoring.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your port and treatment pump

From preparing the line to coming home after disconnection. Your team arranges the appointments and shows you what to do.

1 · Before treatment

Choose and prepare the line

The team explains whether a port or peripherally inserted central catheter (PICC) suits your treatment. You receive instructions for the insertion appointment.

2 · Insertion and recovery

A separate appointment may be needed

For a port, numbing medicine is used and a small device is placed under the skin. Ask about sedation, transport and wound care.

3 · Day 1

The nurse connects your pump

After the day-ward medicines, the nurse checks and connects the fluorouracil pump. Ask for a demonstration before leaving.

4 · At home

Treatment continues for about 46 hours

Carry the pump as shown. Keep the dressing dry and protect the line from pulling, heat and damage.

5 · As arranged

Return for disconnection

A trained professional checks the infusion, disconnects the pump and cares for the line. The appointment is based on connection time.

6 · Between cycles

The access device usually stays

A port needle can be removed while the port stays under the skin. A PICC remains in the arm with ongoing dressing and line care.

Port or PICC: what is the difference?

A port is a small chamber beneath the skin, usually on the chest, attached to a tube ending in a large vein. A special needle goes through the skin into the port when it is used. A PICC enters a vein in the upper arm; a short section remains outside under a dressing. Both can deliver a home infusion. Your team helps choose the device and teaches its care.

Before insertion and during recovery

Tell the team about allergies, blood thinners, infections and any other cancer medicines. Follow the instructions about eating, drinking and transport; do not stop medicines yourself. Port placement uses local anaesthetic, sometimes with sedation or general anaesthetic. The team explains the procedure and risks, including infection, bleeding, clots and insertion complications. Ask how to manage discomfort, protect the wound and resume activity. Report increasing pain, redness, swelling, discharge or fever.

Connection: what the nurse does

The nurse checks the line and treatment, secures the connection and dressing, and starts the pump. Before leaving, you should have the disconnection location and time, contact numbers including out of hours, and the written spill instructions. Ask to be shown what normal pump progress looks like. You do not need to calculate a rate.

Carrying it, sleeping and washing

Use the supplied carrier and the position recommended for your pump. Keep tubing free from pulling and kinks. Ask how to place the pump safely for sleep and how to protect the line and dressing when washing. Do not immerse it or swim. Avoid direct heat and cold. An elastomeric pump has a balloon that shrinks slowly and may have no alarm; an electronic pump has different instructions. Follow the instructions for the device you actually receive.

Disconnection is not port removal

Keep the appointment even if the pump looks empty, unless the team changes the plan. Staff check the pump and line and arrange safe disposal. After flushing and other line care as required, the port needle may be removed and a small dressing applied. The implanted port stays in place until a separate removal procedure is arranged. A PICC remains in place and needs scheduled dressing and flushing care.

Leakage, disconnection or unexpected pump progress

Call the team immediately if the pump leaks, is damaged, finishes unexpectedly early, appears not to run, or the line disconnects. Follow the emergency or spill steps you have been specifically taught, including use of a clamp if instructed. Do not reconnect, change settings or disconnect the pump yourself. Keep other people away from leaked medicine. If medicine touches skin, rinse it off and wash with soap and water, then seek advice.

Pain, swelling or feeling unwell

Contact oncology immediately for pain or swelling around the line, a wet/loose dressing, redness, fever, shivering or suddenly feeling unwell. A painful swollen arm or neck needs urgent advice. Severe chest pain, major breathing difficulty or collapse: call 112 / 999. Do not wait for pump disconnection or the next clinic visit.

The drawings show general steps, not instructions for inserting or accessing a line. Your hospital confirms its own arrangements and the care instructions for your device.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before treatment

Confirm the treatment aim, duration, DPD result, blood tests, prescription and contact numbers.

STEP 2

Visits and access

Day 1: folinic acid and fluorouracil injection, followed by a 46-hour fluorouracil pump. Repeat every 14 days. Adjuvant treatment: up to 12 cycles; advanced treatment: continue only while appropriate. Confirm line access, pump changes and disconnection arrangements.

STEP 3

If treatment is delayed

Ask for revised written treatment and support-medicine instructions. Do not make up missed doses or carry an old plan into a new phase.

STEP 4

Getting help

Keep the wallet card and medicine list available. Contact the team promptly for infection, dehydration, chest symptoms or significant toxicity.

Tests and safety

DPD before fluorouracil

Low DPD activity can cause severe toxicity. Testing helps guide the plan but cannot prevent every reaction. Unexpected severe early symptoms need urgent advice.

Blood tests

Before each 14-day cycle: blood count, renal and liver profile; baseline DPD. Scans/markers follow the consultant plan.

Medicines and interactions

Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment pattern	 **Days 1–14** Day 1: folinic acid and fluorouracil injection, followed by a 46-hour fluorouracil pump. Repeat every 14 days. Adjuvant treatment: up to 12 cycles; advanced treatment: continue only while appropriate.
Symptoms and infection	 **Days 1–14** Report on any day; timing varies

The written plan comes first: Day 1: folinic acid and fluorouracil injection, followed by a 46-hour fluorouracil pump. Repeat every 14 days. Adjuvant treatment: up to 12 cycles; advanced treatment: continue only while appropriate.

Symptoms can continue: Contact oncology for fever, sudden illness, diarrhoea, mouth soreness or inability to drink, during treatment or the break.

> **Why these bands stay broad:** Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00660 v2b official regimen ↗

> **There are no guaranteed "safe days".** Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Between-treatment notes

Day 1: Day-ward medicines and pump connection

Confirm the 46-hour finish time and disconnection appointment. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Do not change pump settings or disconnect it yourself unless specifically trained and instructed.

Day 2–3: Home infusion until 46 hours

Report leakage, disconnection, line pain or a pump that stops unexpectedly. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Do not change pump settings or disconnect it yourself unless specifically trained and instructed.

Day 4–14: Between treatment visits

Record symptoms and confirm blood tests before the next cycle. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Do not change pump settings or disconnect it yourself unless specifically trained and instructed.

Support around the medicines

1 **Supportive medicines**

Use the prescribed anti-sickness and diarrhoea plan; check the medicine, timing and rescue duration. Do not duplicate steroids or anti-sickness medicines.

2 **Mouth, bowel and skin care**

Report soreness and diarrhoea early. Use gentle mouth and skin care and ask for help with food or fluids.

3 **Medicine checks**

4 **Pump and line support**

Confirm the pump-care instructions and a working contact number. Do not adjust, reconnect or disconnect the pump unless specifically trained and instructed.

Recognise symptoms and act

Mouth / eating

Report painful ulcers or difficulty eating and drinking.

Chest / heart

New chest pain needs urgent assessment, including during an infusion.

Bowel / hydration

Report diarrhoea, increased stoma output, severe pain, blood or dehydration.

Hands / feet

Report painful redness, cracking or peeling that affects daily activity.

Blood / infection

Fever, shivers or sudden illness need immediate oncology advice.

Pump / line

Report leakage, line pain, swelling, disconnection or a stopped pump immediately.

Daily life and support

Food and fluids

Aim for tolerable meals and fluids. Report weight loss or poor intake; ask for dietetic support.

Activity and rest

Adjust activity and rest to your energy. Report fatigue that limits normal tasks.

Fertility and sexual health

Ask about fertility preservation before treatment and the contraception and breastfeeding plan for all medicines. Report a possible pregnancy promptly.

Support for you and your family

Ask about counselling, psycho-oncology, practical support, transport and financial advice. Bring someone to discussions if helpful.

Your home infusion

Before leaving

Check the exact finish or change time, disconnection appointment, line care and 24-hour contact route.

If something goes wrong

Report leakage, swelling, line pain, disconnection, an alarm or unexpected pump speed. Follow the supplied spill instructions; do not handle leaked medicine with bare hands.

Changes to treatment

Do not restart, reconnect or change the pump unless specifically trained and instructed. The team updates the prescription and pump plan.

Questions worth asking

Which treatment plan applies to me?

Day 1: folinic acid and fluorouracil injection, followed by a 46-hour fluorouracil pump. Repeat every 14 days. Adjuvant treatment: up to 12 cycles; advanced treatment: continue only while appropriate. Your team confirms any modification.

Do more side effects mean treatment is working?

No. Side effects do not reliably measure response. Report symptoms and follow the agreed review plan.

Who do I contact overnight?

Ask the team to replace example numbers with working daytime, out-of-hours and backup contact routes before leaving.

> **Try explaining the plan back:** Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Fluorouracil treatment day

Active medicines: Folinic acid + Fluorouracil

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before the fluorouracil-only treatment visit NCCP NCIS V8 low-risk default. Review QT risk, interactions, constipation and hepatic function; clinician may select an alternative.
Metoclopramide	10 mg orally up to three times daily as required	Rescue; prescriber confirms duration Check renal function, neurological contraindications and interactions.

Additional phase-specific support

- Use the prescribed anti-sickness and diarrhoea plan; check the medicine, timing and rescue duration. Do not duplicate steroids or anti-sickness medicines.
- Report soreness and diarrhoea early. Use gentle mouth and skin care and ask for help with food or fluids.
- Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.
- Confirm the pump-care instructions and a working contact number. Do not adjust, reconnect or disconnect the pump unless specifically trained and instructed.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____